F09 · 24M, infected L forearm laceration with cellulitis, schizophrenia off meds with command hallucinations and aggression toward staff, unhoused

Fit score **15/100** · Likely not a fit · blockers: MH-1, MH-3, OTH-1, FORM-X7 · Claude's read 3/100 (deny) · 26/26 quotes verified on their pages

IHH admission criteria (32 applicable, 30 not applicable or superseded)

Rule	Requirement	Status	Finding	Pages
General				
GEN-1	Unhoused and too ill to recover	Met	Unhoused with an infected wound needing antibiotics and suture follow-up.	p4✓, p3✓
GEN-3	Re-hospitalization risk	Met	Authorization request marks him at risk of re-hospitalization.	p3✓
ADLs				
ADL-2	Independent hygiene	Unknown	Independent bathing is not specifically documented.	–
ADL-3	Independent toileting	Unknown	Independent toileting is not specifically documented (form says continent).	–
ADL-1	Independent ADLs	Met	Fax cover states ambulatory and independent; the chart does not describe ADL assistance.	p1✓
Mobility				
MOB-2	PT/OT notes sent	Not met	No PT/OT notes are in the packet.	–
MOB-3	Walks 100 ft	Unknown	Form says ambulatory, but 100 ft distance is not documented.	–
Mental Health				
MH-1	Mentally stable on meds	Not met	Acute schizophrenia exacerbation; risperidone only restarted 09/04 and psychiatry recommends stabilization before any congregate setting.	p8✓, p8✓
MH-3	No command hallucinations	Not met	Command auditory hallucinations to harm others are documented.	p8✓, p7✓
MH-5	No conservatorship/FSP	Unknown	Conservatorship/FSP is not mentioned; a 5150 evaluation is pending daily reassessment.	–
MH-2	No suicidal ideation	Met	Denies suicidal ideation to psychiatry.	p8✓
Cognition				
COG-1	Self-administers meds	Unknown	Form says he self-administers all meds, but he stopped risperidone for 3 months and is refusing care; ability to manage meds is not confirmed.	–
COG-2	AOx4	Met	AOx4 on the 09/05 progress note and psychiatry exam.	p9✓
Other Factors				
OTH-1	Not aggressive	Not met	Threatened and threw a tray at an RN on 09/03 and was verbally aggressive toward a CNA on 09/04.	p7✓, p7✓
OTH-4	Not on isolation	Unknown	Isolation is not mentioned (MRSA negative; on 1:1 sitter for behavior, not isolation).	–
OTH-2	Sex offender case by case	Met	Referral form says he is not a registered sex offender.	p2✓
Infection Control				
INF-2	COVID swab within 24h	Not met	No COVID swab: patient refused on 09/05.	p10✓
INF-1	MRSA swab negative	Met	MRSA nares PCR negative 09/01.	p10✓
Infection				
ID-2	Takes oral antibiotics	Unknown	Form says he self-administers meds, but recent nonadherence makes independent oral antibiotic use unconfirmed.	–
ID-Q3	Is the patient on isolation?	Unknown	Isolation not mentioned.	–

Rule	Requirement	Status	Finding	Pages
ID-1	Last 5 vitals sent	Met	Last vital sets sent (one refused); afebrile, HR 88-96, normal BP, no signs of sepsis.	p10✓
ID-Q1	Location of infection?	Met	Left dorsal forearm laceration.	p6✓
ID-Q2	Were cultures performed? Results shoul...	Met	Wound culture grew pan-sensitive group A strep.	p10✓
Integumentary				
INT-E2	Wound care to be coordinated with home...	Unknown	Home health not ordered; form says independent with wound care, but dressing-change frequency is not stated.	–
INT-E1	Uncomplicated infections of the skin,...	Met	Cellulitis improving on oral antibiotics (cephalexin through 09/08).	p9✓
INT-E5	Post-surgical incision. If staples or...	Met	Sutures in place with removal scheduled 09/11 at the wound clinic.	p2✓
INT-X2	Complicated large abscess	Met	Cellulitis after laceration, not a complicated large abscess; no foreign body.	p10✓
Referral form				
FORM-X7	Not aggressive (form)	Not met	Aggressive and combative toward nursing staff 09/03 and 09/04.	p7✓, p8✓
FORM-X5	Not refusing detox	Unknown	Methamphetamine and cannabis use documented, but no detox offer or refusal to abstain is recorded.	–
FORM-X1	Not cognitively impaired	Met	Cognition grossly intact, AOx4; not cognitively impaired.	p8✓
FORM-X2	Can do ADLs	Met	No ADL deficit documented; fax says independent.	p1✓
FORM-X4	No uncleared MRSA/TB/C-diff	Met	Infection is strep cellulitis, not MRSA, TB or C. diff; MRSA PCR negative.	p10✓

IHH document checklist

Required document	In packet	Pages
DOC-1 Illumination Health + Home Referral Form	Yes	p2
DOC-2 Client's Demographic Page	Yes	p4
DOC-3 CalOptima Form/LOA/AUTH	Yes	p3
DOC-4 H&P (History and Physical)	Yes	p5, p6
DOC-5 Current Progress Notes (if needed)	Yes	p8, p9
DOC-6 Updated Vitals (last 24 hours)	Yes	p10
DOC-7 Medication List	Yes	p11
DOC-8 PT/OT or Nursing Notes	Yes	p7
DOC-9 Blood or Wound Culture (if needed)	Yes	p10
DOC-10 30-day supply of all medications	Unknown	–

Questions for the hospital

1. Has psychiatry cleared him as stable for a congregate setting, and has he had any aggression or command hallucinations since starting risperidone 09/04?
2. Is he still on a 1:1 sitter, and what is the outcome of the daily 5150 reassessment?
3. Could you please send a negative COVID swab taken within 24 hours of arrival?
4. Will the wound need dressing changes before the 09/11 suture removal, and who will do them?
5. Can you confirm he will take cephalexin and risperidone on his own, and will a 30-day supply be sent?

AI-prepared checklist. The coordinator decides. ✓ = the quoted text was found on that page of the transcript; (check) = not found, read the page. Fit score weights were hand-tuned on IHH's 11 past decisions (in-sample).